

Transforming Certification for Better Care

ABMS CONFERENCE 2026

ABEM's MyEMCert:
Learning through Key Advances

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Disclosure

- Kevin Joldersma is an employee of the American Board of Emergency Medicine

Why ABEM Replaced ConCert

The Challenge

- Traditional 10-year ConCert examination was:
 - Episodic
 - High stakes
 - Limited educational value
 - Less aligned with lifelong learning

Stakeholder Priorities

ABEM's 2017 specialty-wide survey and stakeholder engagement identified preferences for:

- Smaller, more frequent assessments
- Open-book administration
- Immediate feedback
- Greater clinical relevance
- Inclusion of emerging evidence
- Learning-oriented assessment processes

MyEMCert Structure

Core Components

- 4 modules every 5 years
- Open-book
- Immediate feedback
- Modular content
- Core Content questions
- Case Series questions
- Key Advances questions

Key Design Goal

Integrate certification, learning, and knowledge translation into one activity.

ABEM's Article-Based Assessment Model: Key Advances

Built from:

- Practice-changing literature
- Clinical policy updates
- Emerging evidence
- Curated evidence summaries (synopses)

Questions are designed to:

- Promote application of evidence
- Highlight important developments in EM
- Translate literature into practice

Not designed to:

- Test article memorization
- Reward recall of publication details

Instead they emphasize:

- Interpretation
- Clinical application
- Decision making

Validity Evidence Framework

ABEM has accumulated evidence from:

1. Stakeholder input
2. Content-development processes
3. Psychometric performance
4. Response-process analyses
5. Post-module survey evidence
6. Longitudinal monitoring

This follows contemporary validity frameworks emphasizing:

- Content
- Response process
- Internal structure
- Consequences of testing

Validity Evidence: Physician Perceptions

2021 Survey vs. 2025 Survey

Question	2021 Survey	2025 Survey
Reflects current EM topics	90.4%	86.2%
Important to clinical practice	89.2%	85.0%
Assisted learning	82.7%	80.8%
Assesses clinical decision making	82.7%	79.1%
More likely to apply Key Advances	73.6%	71.7%

Why These Survey Results Matter

Response Consequences as Validity Evidence

These data represent evidence related to the consequences of testing.

ABEM designed MyEMCert to:

- Promote learning
- Increase relevance
- Encourage evidence uptake

The survey findings demonstrate that physicians continue to perceive those intended outcomes years after implementation.

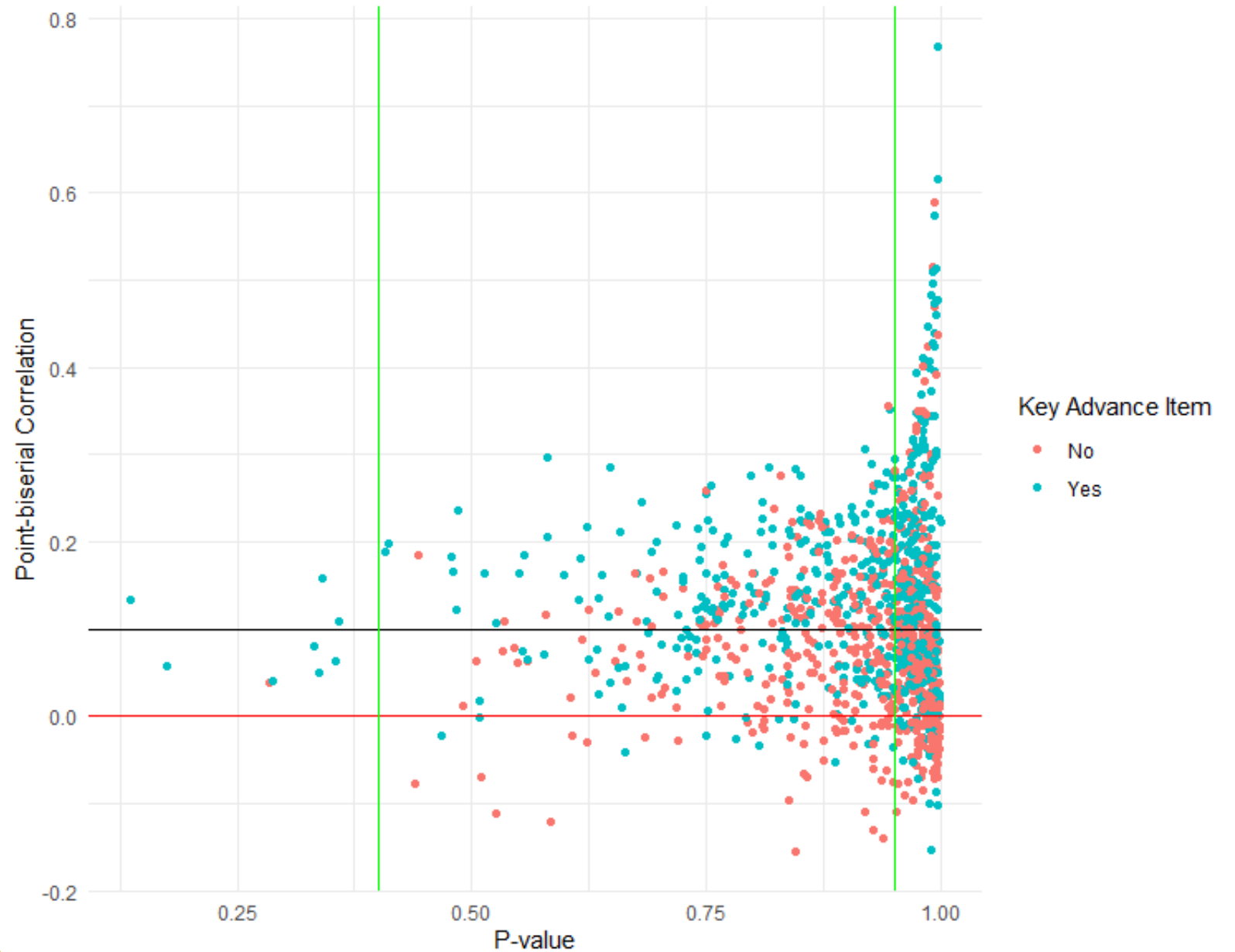
Key Advances as a Knowledge Translation Tool

Most Important Finding

- 71.7% of physicians in 2025 reported being “more likely to apply Key Advances to patient care after participation”.
- This compares favorably to the 73.6% endorsing in the 2021 survey.

Item Analysis Results (2025)

Metric	Non-Key Advance	Key Advance	Difference
Mean Difficulty (P-value)	0.91	0.89	-0.02
Mean Point-Biserial	0.09	0.15	+0.06



From Emerging Evidence to Clinical Application

- **Identify important evidence**
 - Practice-changing research
 - Clinical policy updates
 - Emerging evidence relevant to emergency medicine
- **Curate the evidence**
 - Select findings with potential implications for practice
 - Develop a concise synopsis
 - Focus attention on the clinically actionable message
- **Develop the assessment**
 - Write questions requiring interpretation and application
 - Avoid testing article trivia or publication details
 - Provide immediate feedback and an explanation of the correct answer
- **Support knowledge translation**
 - Reinforce the evidence through active decision making
 - Encourage physicians to consider incorporating the finding into practice
 - Meet physicians where they are at with videos, synopses, podcasts and more

Article-Based Assessment Changes the Response Process

Key Advances ask physicians to do more than recall information

- Review a curated summary of emerging evidence
- Determine which findings are clinically relevant
- Interpret the implications for a patient or practice decision
- Apply the evidence to an assessment question
- Receive immediate feedback that reinforces the intended interpretation

Evidence of a distinct response process

Item Type	Mean Response Time
Non-Key Advance	81.9 seconds
Key Advance	89.1 seconds
Difference	+7.2 seconds

Do ABEM's Article-Based Key Advances Accomplish Their Intended Purpose?

Intended Outcome	Evidence Presented
Focus attention on current evidence	86.2% reported that content reflects current EM topics
Emphasize clinically important information	85.0% reported that content is important to clinical practice
Support learning	80.8% reported that the assessment assisted learning
Assess application and judgment	79.1% reported that it assesses clinical decision making
Encourage knowledge translation	71.7% reported being more likely to apply Key Advances to patient care
Function as an assessment	Key Advance items demonstrated slightly greater average discrimination than non-Key Advance items

Five Design Principles for Article-Based Assessment

1. Curate, rather than simply assign, literature

Select evidence for clinical importance and provide a concise synopsis that reduces unnecessary information burden.

2. Assess application, not article recall

Questions should require interpretation, clinical reasoning, and decisions rather than recall of authors, methods, or publication details.

3. Make feedback part of the intervention

Immediate feedback and correct-answer explanations connect the assessment decision to the evidence.

4. Evaluate both assessment and learning outcomes

Monitor item performance, response behavior, physician perceptions, and anticipated use of evidence in practice.

5. Treat the model as continuously improvable

Evaluate synopses, rationales, audiovisual materials, item design, and delivery features empirically.

ABEM's Article-Based Assessment Connects Certification, Learning, and Practice

Current evidence

Key Advances allow MyEMCert to incorporate clinically important emerging literature into an ongoing certification program.

Active learning

Curated synopses, application-focused questions, and immediate feedback turn evidence review into active clinical decision making.

Knowledge translation

In the 2025 survey, **71.7%** of physicians reported that participation made them more likely to apply Key Advances to patient care, closely replicating the **73.6%** reported in 2021.